

METROPOLITAN GENERAL HOSPITAL

Department of Internal Medicine — Admission Note

Patient Information

Patient Name:	John Michael Carter	MRN:	MGH-2024-087432
Date of Birth:	14 March 1968 (Age: 57)	Gender:	Male
Admission Date:	03 June 2026 09:42 AM	Ward:	4B — General Medicine
Attending Physician:	Dr. Priya Nair, MD	Admitting Resident:	Dr. Arjun Mehta, MD
Insurance:	BlueCross BlueShield (PPO)	Allergies:	Penicillin (rash), Sulfa drugs

Chief Complaint

Mr. Carter is a 57-year-old male presenting with a 3-day history of progressively worsening shortness of breath, bilateral lower-extremity pitting edema, and an unproductive cough. He reports orthopnea (requiring 3 pillows to sleep) and paroxysmal nocturnal dyspnea.

History of Present Illness

The patient was in his usual state of health until approximately 72 hours prior to admission, when he began to notice increasing dyspnea on exertion with minimal activity (walking to the bathroom). He denies chest pain, palpitations, syncope, or recent upper respiratory tract infection. He reports a 6 kg weight gain over the past two weeks despite maintaining a low-sodium diet. He endorses fatigue and reduced urine output. He had a similar episode 18 months ago requiring hospitalization and responded well to IV diuresis.

Past Medical History

- Congestive Heart Failure (EF 35%, last echo Jan 2026)
- Type 2 Diabetes Mellitus — on metformin, well-controlled (HbA1c 6.8%)
- Hypertension — on lisinopril 10 mg daily
- Hyperlipidaemia — on atorvastatin 40 mg nightly
- Chronic Kidney Disease Stage 3a (baseline Cr 1.4 mg/dL)

Current Medications

Medication	Dose	Route	Frequency
Furosemide (Lasix)	40 mg	PO	Once daily (AM)
Lisinopril	10 mg	PO	Once daily
Carvedilol	12.5 mg	PO	Twice daily
Metformin	500 mg	PO	Twice daily with meals
Atorvastatin	40 mg	PO	Nightly
Aspirin	81 mg	PO	Once daily
Spironolactone	25 mg	PO	Once daily

Physical Examination

Vital Signs:	BP: 158/94 mmHg HR: 98 bpm (regular) RR: 22/min SpO2: 91% on room air Temp: 36.8°C Wt: 94 kg Ht: 175 cm BMI: 30.7
General:	Alert, oriented x3, mildly dyspnoeic at rest, speaking in short sentences, no acute distress.
Cardiovascular:	S1/S2 present; S3 gallop appreciated at the apex. JVD at 10 cm H2O. No murmurs.
Respiratory:	Bilateral basilar crackles on auscultation, dullness to percussion at lung bases. No wheeze.
Abdomen:	Soft, non-tender, non-distended. Hepatomegaly palpable 3 cm below costal margin.
Extremities:	2+ pitting oedema bilaterally to the knees. Peripheral pulses 2+ and equal.
Neuro:	CN II–XII grossly intact. Strength 5/5 throughout. No focal neurological deficits.

Assessment and Plan

Primary Diagnosis: Acute decompensated heart failure (HFrEF, EF 35%) — likely precipitated by dietary sodium indiscretion and medication non-compliance.

1. Admit to telemetry ward; continuous pulse oximetry and daily weights.
2. IV furosemide 80 mg BID; strict fluid restriction 1.5 L/day; daily electrolytes (K+, Na+, Cr).
3. Cardiology consultation requested — consider repeat echocardiogram.
4. Continue carvedilol and lisinopril; hold metformin pending renal function trend.
5. Deep vein thrombosis prophylaxis: enoxaparin 40 mg SC daily.
6. Patient education: low-sodium diet (<2 g/day), daily self-weighing, medication adherence.
7. Goals of care discussion with patient and family to be arranged by Day 2.

Electronically signed by: Dr. Arjun Mehta, MD (PGY-3)

Date/Time:

03 Jun 2026 | 10:15 AM

Reviewed & Approved: Dr. Priya Nair, MD — Attending

Pager:

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